

# The sociology of healthcare professions: the case of the professionalization of psychology in Spain

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In this study, we analyse the discursive strategies of professionalization used by psychology in Spain during its professionalization phase in the health sector in the early twenty-first century. Our primary focus is the conflict with political regulators and other healthcare professions after the *Ley de Ordenación de Profesiones Sanitarias* (Healthcare Professions Act) came into effect, since this Act calls into question the extent to which psychology can be viewed as health care. We use discourse analysis to determine the discursive structure of the conflict, which has an interprofessional dimension, as well as the professionalization and social closure strategies of psychology within the framework of professional relationships (boundary work). We argue that understanding this conflict requires viewing it as part of the European and international regulation context of healthcare professions. Highlighted in the analysis is the emergence of two psychology discourses divided into four modes of enunciation (professionalizing, cultural, scientific, and political–economic). Among other aspects, the conclusions focus on the dual interprofessional and intraprofessional nature of the conflict and evidence of a dual closure in psychology. The latter's professionalization strategies in the sector occasionally resemble those employed by nursing, although at other times they are similar to those followed by some paramedical professions.

**KEYWORDS:** health psychology; LOPS; professionalization; new public management; boundary work; professional regulation.

## INTRODUCTION

Several studies have analysed the regulation processes of healthcare professions from the perspective of the sociology of professions. These studies help us indicate that international regulation processes have followed an extremely similar direction in recent decades (Saks 2014; Leslie et al. 2021; Rowland, Albert and Kitto 2021; Adams 2022). The most usual arguments for these regulations are the requirements to modernize the sector; better structuring of increasingly complex and interprofessional organizational environments; seeking more protection for users or customers; the expansion of accountability mechanisms and transparent professional practice (audit culture); general improvement in the quality of this practice; establishing professional standards of practice; and controlling expenditure and tax inefficiency in the health sector. These regulation processes have led to several wide-ranging consequences for a variety of healthcare

professions. In the case studied here, our interest lies in outlining the consequences for one of these professions—psychology—specifically for its ongoing professionalization process in the Spanish public healthcare sector.

As mentioned above, there is extensive literature on recent regulation processes of healthcare professions, although the vast majority of these studies focus on Anglo-American countries (UK, USA, Australia, and Canada). Some, albeit fewer, studies highlight the case in European countries (King et al. 2015; Almeida and Gabe 2016; Rowland, Albert and Kitto 2021). Hardly any of these regulatory processes have been conducted from the perspective of the sociology of professions in the twenty-first-century Spanish context (Giarelli and Saks 2024). Furthermore, when recent studies focus on a specific profession, this mainly tends to be that of a physician or nurse (King et al. 2015; Feyereisen and Goodrick 2019).

Sometimes, alternative medicine or ‘paramedical’ professions, such as chiropractors or osteopaths, are the focus (Saks 1992; Brosnan 2017). Very few studies concentrate specifically on psychologists, and those that do tend to only include them in a framework of analysis of a broader spectrum of professions (Goode 1960; Le Bianc and Svensson 2008; Bucher et al. 2016; Fincham and Forbes 2019).

This work is, therefore, a case study of the Spanish regulation of psychology as a healthcare profession (working in mental health) from the perspective of the sociology of professions, specifically the professionalization processes of that discipline. The last key moment marking this professionalization process in Spain is the conflict that arose with Spanish psychology when the *Ley de Ordenación de Profesiones Sanitarias* (Healthcare Professions Act), known in Spain as the LOPS, came into effect.

The largest protests in the recent history of Spanish psychology occurred between 2004 and 2005. They were a direct response to the new regulation of healthcare professions in Spain as a result of the LOPS (Boletín Oficial del Estado 2003). In accordance with LOPS regulations, psychology was not fully recognized as a healthcare profession, causing alarm both in the discipline’s university education area and in its professional sphere, which was experiencing an ongoing consolidation process. At that time, only one healthcare speciality was recognized in Spain for psychology through the ‘Official Qualification of Psychologist Specializing in Clinical Psychology’ (1998). The issue was that this qualification could only be secured after obtaining a degree through a residency process (PIR, Psychologist in Residence) lasting four years. Furthermore, the number of places offered across Spain was limited. In fact, even today, Spain still only has 5.4 psychologists for every 100,000 inhabitants in the public health service. This ratio is the lowest in European countries for which there is data (Civio 2021). Another complaint about the new Act was the lack of a general regulatory mechanism, which was not established by the LOPS. After some years, new regulations were incorporated for psychology, primarily a master’s degree in General Health Psychology, which has solved part of the problem. However, unease in the discipline has not abated. We argue that the decisive moment in this series of unresolved difficulties lies in the aforementioned conflict with the LOPS, which is why we analyse it in depth here. It is also important to clarify that providing professional healthcare through mental health work is not psychology’s only focus. As an occupational activity, psychology also appears in other contexts, for example, education, business (primarily in human resources), council social work, and non-governmental organizations. Nevertheless, the

healthcare aspect predominates in the professional profile of psychology in Spain, given that almost 70 per cent of the country’s psychologists are employed in this sector (Santolaya, Berdullas and Fernández 2002). This area of psychology also has the strictest regulation and training processes, and most of Spain’s students studying a degree in psychology set their sights on it.

We analyse the discourse concerning the LOPS to study the conflict and mobilizations in psychology after the Act was published. Studies of the sociology of professions afford a wealth of resources for analysing psychology’s conflict with the LOPS, since they provide an exceptional perspective on the historical and contemporary contexts defining how certain occupations are (or are not) considered as professions. They also highlight the implications associated with this consideration. Some of the relevant concepts and studies include professionalization and its strategies. The conflict caused by the LOPS also concerns other healthcare professions; therefore, the possibilities of professionalizing psychology depend on how it relates to and negotiates with other professions in the sector, primarily physician-psychiatrists. In this case, the sociology of professions also affords extensive literature on interprofessional relations and boundary work, which serve as a framework for understanding the analysed conflict.

Finally, placing these interprofessional tensions in the sector within the context of new international public sector regulations is also relevant. The emergence of the new public management, which affects the health sector, is an unavoidable framework for the conflict due to its potential as a source of new professionalization possibilities. However, it is also a source of possible loss of the ‘classic’ characteristics of the autonomy, privileged social position or self-regulation of healthcare professions, especially medicine, stemming primarily from the introduction of managerial logic. This process has resulted in several debates on the sociology of professions, dominated by the concept of ‘hybridized professionalism’.

We first review the framework of studies on the sociology of professions and then describe the specific area of regulation of healthcare professions in Spain brought about by the LOPS and the ensuing conflict with psychology. To properly put this regulation into context, we briefly examine how this regulation materialized in other European countries at similar times and in similar yet different forms. We specifically explore the cases of the UK and France. The context of conflicts with psychology in both countries helps us assess the particular nature of the discursive modes of psychology’s response in Spain.

After properly contextualizing the conflict, we finally analyse it through a discursive analysis (documentation

and semi-structured interviews) of all the collective agents directly involved in the conflict. These agents mainly stem from the psychological sphere (professionals, academics, and students) but also from other participating professional collectives (mainly physician-psychiatrists) and from the institutional-state agents who promoted the Act or its modifications. As mentioned above, the aim is to identify psychology's (discursive) professionalization strategies in this changing regulatory framework and within the context of conflicts or negotiations in the Spanish interprofessional healthcare field.

### PROFESSIONALIZATION, BOUNDARY WORK, AND NEW PUBLIC MANAGEMENT

The classic functionalist perspective considered professional skill as the preserve of a series of occupations whose functional role in the social structure led to more far-reaching roles and statuses (Parsons 1939). Rather than being perceived as part of an almost 'natural' dimension of society's structural needs, the profession is never the definitive result of a complex path of struggles and conflict, in which professional groups are subject to political, cultural, academic or economic variables that do not entirely depend on them. Consequently, they can never be fully or permanently independent or self-regulating. Instead, at most, professions can find themselves at different points or phases of professionalization.

The wide variety of professionalization mechanisms replacing or complementing the classic professional criteria (altruistic service, ethical regulating code, technical skill, and professional authority) generally include having a systematic and organized body of knowledge and mastery or 'cognitive exclusivity' over legitimized training processes; self-regulated or state mechanisms for professional recognition, accreditation and protection; ability to manage the profession's cultural signifiers; and the establishment of a different product and monopolistic control of the market for its production or sale (Larson 1977; Torstendahl and Burrage 1990; Sáez and Sánchez 2009).

Professional groups begin a social closure when they seek to legitimize their scope and to control or regulate legal or normative access procedures, educational and training processes or ethical limitations. Generally aided or covered by the state, these actions ensure some privileges for the profession (Abbott 1988; Saks 2015). This social closure tends to mobilize resources (e.g. economic, political, social, educational, and ideological) to convince state, public, or professional stakeholders that their distinctive skills or knowledge entitle them to privileges,

including more autonomy in their professional endeavour (Adams 2018).

Professionalization strategies aimed at ensuring social closure often involve participating in a framework of interprofessional negotiation and conflict encompassing several forms of professional closure and boundary work (Gieryn 1983). By boundary work, we understand the 'ideological and rhetorical work that professions use to achieve an apparent distinction of goals, methods, capabilities, and expertise from the other occupants of their ecosystem' (Boussard 2018: 5). These boundaries are also social and symbolic. Symbolic boundaries are conceptual distinctions used to categorize and classify practices, objects, and people (Lamont and Molnar 2002). These symbolic boundaries lead to social boundaries, which are objectified forms of social differences. Fighting for social boundaries is, firstly, often rhetorical, which highlights the importance of studying the discourses used by the professions to protect them and thereby anticipate possible future labour divisions.

Several studies have analysed the discursive strategies that professions use to build or attempt to modify professional boundaries (Abbott 1988; Allen 2000; Timmermans 2002; Suddaby and Greenwood 2005; Bucher et al. 2016; Brydges 2022). In the case of healthcare professions, prior studies have placed importance on two elements in boundary research: centrality and status (Bucher et al. 2016). In the healthcare system, physicians hold the positions with the highest status, which gives them more control over the work of other professions, such as nurses and pharmacists (Freidson 2001). Another relevant distinction differentiates between two areas in the sector based on their centrality: a medical area (including physicians, nurses, and others) and a psychotherapeutic area with psychiatrists at the top, followed by psychologists and other groups, such as social workers (Abbott 1988). In the above framework, we can see that psychology is a lower-status and peripheral profession in the healthcare sector, which conditions the strategic possibilities for professionalization and legitimization. Besides the above, a broader political-economic framework is also a necessary context for understanding the strategic possibilities of professions and their interprofessional tensions (Reed 2018).

A large volume of scientific literature evidences a decline in the self-regulation of professional practice due to marketization, rationalization or political governance processes, or, more recently, in the public sphere, due to new public management (Lane 2000; Freidson 2001; Bezes et al. 2012; Waring 2014; Leitch 2015; Reed 2018). This new public management has involved wider market dominance and discipline concerning the organization

of the jurisdictional domains of professions, now subject to new competitive, audit and accountability regimes and technologies. Widespread fragmentation in the division of expert labour has been the result. As Reed says (2018: 2), ‘under the structural and political pressures of neo-liberalization, the expert division of labour ... has fragmented and fractured so the contradictions between and within expert groups claiming professional authority have become more intense and polarized’. At the same time, the arrival of the managerial focus in healthcare settings has led to conflicts with healthcare professionals, primarily physicians, who have sometimes perceived these changes as a challenge to clinical freedom and their professional autonomy (Waring and Currie 2009; Irigoyen 2011; Siebert et al. 2018).

A rationale with more managerial content was then gradually implemented. This began ideologically in the Spanish healthcare sector with the *Informe Abril* (April Report 1991). It was regularized by Act 15/97 and consolidated by the 2003 healthcare regulation laws. The LOPS, but also the *Ley de Cohesión y Calidad* (Cohesion and Quality Act 2003) and the new Framework Statute for Statutory Employees of Health Services (2003), established a new employment relations model for health services personnel (Rodríguez-López, 2015).

According to the literature (Martín 2008; Irigoyen 2011), these new transformations in the sector in Spain have resulted in losing professional autonomy opportunities. However, this process has also given rise to negotiation dynamics between professionalism and managerialism. Both have been integrated using differing formulas, thereby generating, for example, the well-known hybrid managerial-healthcare roles, which tend to be medical leadership positions (McGivern et al. 2015). Several studies on professions have highlighted the formation of a ‘hybridized professionalism’ in the public sector, in which elements of professional corporate work have been combined with bureaucratic managerial regulation to create new formats of organizational standards and principles for professionals (Freidson 2001; Noordegraaf 2007; Evetts 2011; Heusinkveld et al. 2018).

Lastly, we briefly examine the situation of two countries—the UK and France—which have also been immersed in similar processes of psychology fighting against new healthcare regulations.

## THE NEW HEALTH REGULATIONS IN THE UK AND FRANCE

The professional (and academic) transformation processes affecting psychology extend beyond the frontiers of Spain, and, at the very least, they are markedly

European. The regulations of European countries and recommendations made Europe-wide for the training and accreditation processes of psychology are gradually aligning. Since the end of the twentieth century, the EuroPsy project implemented by the European Federation of Psychologists’ Associations has been taking the steps required to align the training and validate a professional psychologist qualification in Europe (Lunt 2002; Rodríguez-López 2015; Neto et al. 2020). Known as the EuroPsy Certificate, it enables the free movement of professionals in Europe. The gradual internationalization of professional regulations involves considering regulatory frameworks whose reference point is larger geographical areas beyond the national arena (Le Bianic and Svensson 2008). Professional collectives must therefore adapt their professionalization strategies to the new, broader frameworks.

However, in a similar way to the Spanish case, these alignment and regulatory modification processes have not been implemented in other countries without a significant part of psychology raising dissenting voices. We now briefly look at the situation in two European countries, the UK and France, where the mobilizations of psychologists have been more pronounced and have served as a reference in Spain. The comparison helps us narrow down similarities and differences between their healthcare profession regulations and those applied in Spain. Specifically, it sheds light on common ground and divergences found in psychology’s opposition processes to these regulations and, therefore, their discursive strategies of professionalization.

In the UK, the reference regulation is the Health Professions Order, which came into force in 2001. This law created the Health Professions Council (HPC) for regulating fifteen healthcare professions, including psychology. The functions entrusted to the HPC include protecting various qualifications and certificates; creating standards of conduct and training; establishing a public register of professionals; and mediating abuse cases brought by users of the health services these professions provide. In 2008, the Professional Liaison Group was created to regulate health psychology, specifically psychotherapy and counselling. The documents detailing both the standards of professional conduct and the training required for qualification were published in 2009, thereby considerably intensifying critical work on an important section of psychology, ongoing since the Health Professions Order came into force.

The HPC was given the power to approve training courses leading to professional qualifications; establish acceptance criteria in the register of professionals; draw up professional standards (ethics, ‘competence’, and

continuous development); and even to deal with complaints concerning these professionals, which is a good illustration of how crucial this body is. The HPC thus became the main target of criticism voiced by a significant part of psychology in the UK, although some criticism levelled at regularization was made years before it was established (Totton 1999; Postle 2007; Postle and House 2009; Reeves and Mollon 2009). The main objections to the process were the lack of evidence of the need or ability of this type of regulation to protect patients and the de facto medicalization of the sector. It was also pointed out that the criteria and standards were neither neutral nor merely ethical, as they gave preference to a specific type of psychotherapeutic perspective (cognitive behavioural therapy), thus largely resulting in excessive standardization, homogenization, and discrimination. Finally, to a lesser extent, there was some opposition to any form of state regulation on the grounds of freedom of practice. In some psy perspectives, there was also a refusal to even be considered as healthcare professionals, as this was tantamount to a mechanism for adapting to professional medical criteria.

In the case of France, the event that triggered the psychology protests was the approval, in October 2003, of amendment 336 (the third modification of the Public Health Code), popularly known as the Accoyer Amendment. When the French National Assembly voted unanimously for the Accoyer Amendment on 8 October 2003, psychology professionals were excluded from this proposal to regulate the field of mental health (Miller 2005).

The Accoyer Amendment established that psychotherapies should be applied to mental illnesses, and that they should be exercised exclusively by physician-psychiatrists or psychologists with the professional qualifications required by the decree. In this case, the aim of the new amendment was specific regulation of psychotherapy (therapies associated with mental health) and not of healthcare professions as a whole, as in the Spanish and UK cases. The reason for the new law was similar to the UK case (and also partly to Spain's), especially the need to protect psychotherapy users and to promote sector quality by protecting a qualification that prevents the profusion of 'sham' or sectarian practices.

Psychology's response to the Accoyer Amendment was not long in coming. In this case, it was psychoanalysis, with Lacan's well-known son-in-law (Jacques-Alain Miller) at its head, which led the opposition to the amendment, appealing for the need for a coalition of psychoanalysts, psychotherapists, and psychologists, known as the 'psy movement' (Miller 2003). Protests grew steadily in the following years, focusing on two general demands: firstly,

criticism of the underlying medicalization of regulation and the resulting forced submission of psychologists to psychiatrists; and, secondly, the denouncement of a process of psychological university sanitization of therapies and training alternative to the dominant model (Enriquez 2004; Roudinesco 2004; Champion 2006).

## METHODOLOGY

We studied the discourse concerning the LOPS to analyse the conflict and the mobilizations generated in psychology after the Act was published. For the analysis, we collated a total of thirty-six public documents in both paper and digital formats. The documents selected are official reports or communiqués of the professional, academic and student representative bodies of psychology, of numerous collectives and associations involved in this conflict and official reports or communiqués by the Ministry of Health and bodies linked to it (advisors). The documents were mainly taken from the official journals of these collectives or from their websites.

The selection process of the collectives was always based on their participation in the public debate on the LOPS. Consequently, all the collectives participating in forums, areas of debate and public intervention, or mentioned in them, always in connection with the specific subject of psychology, were gradually incorporated one by one into the analysis. As a result, there was no a priori selection of the collectives; instead, they were chosen based on the public LOPS/psychology discourse. Given that professional collectives that were not specifically concerned with psychology also intervened in this debate (mainly physician-psychiatrists), their discourse was also collated and analysed. In addition, we decided to include the study of the discourse of student collectives. Despite not being professional collectives, they did participate intensely in the public debate and also demonstrated the importance of the academic-university sphere in this professional conflict. We were also interested in their discourse to possibly anticipate the discursive logic of future psychology professionals. Table 1 shows the collectives whose documents were involved in the collection and analysis process.

Besides analysing public documentation, fourteen semi-structured interviews were conducted with privileged sources (experts) and with prominent figures directly involved in the conflict. These figures generally hold important positions in the collectives participating in it (physicians involved in the initial drafting of the Act; physicians or psychologists who were members of the policy negotiation committees for amendments to the Act; senior management of the organizations for the

**Table 1.** Groups analysed in the study.

| Groups                                                                                                                                                                                                   |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Foro de la Psicología de España [Spanish Psychology Forum]                                                                                                                                               |
| Consejo General de Colegios Oficiales de Psicólogos (CGCOP) [General Council of Official Colleges of Psychologists]                                                                                      |
| Conferencia de Decanos de Psicología de las Universidades Españolas [Conference of Deans of Psychology of Spanish Universities]                                                                          |
| Colectivo de Estudiantes de Psicología (CEP-PIE) [Psychology Students' Collective]                                                                                                                       |
| Movimiento contra la LOPS [Movement against the LOPS]                                                                                                                                                    |
| Grupo por la salud y la psicología [Group for Health and Psychology]                                                                                                                                     |
| Asociación de Psicólogos Afectados/as por el Reconocimiento de la Especialidad en Psicología Clínica [Association of Psychologists Affected by the Recognition of the Speciality in Clinical Psychology] |
| Federación Española de Asociaciones de Psicoterapeutas (FEAP) [Spanish Federation of Psychotherapists' Associations]                                                                                     |
| COP Andalucía Occidental [Official College of Psychologists of Western Andalusia]                                                                                                                        |
| COP Galicia [Official College of Psychologists of Galicia]                                                                                                                                               |
| Asociación Española de Psicología Clínica y Psicopatología (AEPCP) [Spanish Association of Clinical Psychology and Psychopathology]                                                                      |
| Asociación Nacional de Psicólogos Clínicos y Residentes (ANPIR) [National Association of Clinical and Resident Psychologists]                                                                            |
| Asociación Española de Neuropsiquiatría (AEN) [Spanish Association of Neuropsychiatry]                                                                                                                   |
| Mesa Galega de Psicología Clínica (MGPC) [Galician Board of Clinical Psychology]                                                                                                                         |
| Sociedad Española de Psiquiatría [Spanish Society of Psychiatry]                                                                                                                                         |
| Consejo General del Colegio de Médicos [General Council of the Medical Association]                                                                                                                      |
| Consejo Asesor del Ministerio de Sanidad [Advisory Council of the Ministry of Health]                                                                                                                    |
| Comisión Nacional Promotora de la Especialidad de Psicología Clínica (CNEPC) [National Committee for the Promotion of the Speciality of Clinical Psychology]                                             |

**Table 2.** Interviews conducted in the study.

| Academic     | Clinical               | Educational | Social       |
|--------------|------------------------|-------------|--------------|
|              | E4 E3 E10 E13          | E14 E12     | E7 E11 E9 E8 |
| Professional | Physician–psychiatrist |             | Psychologist |
|              | E1 E5                  |             | E3 E2 E4 E6  |
| Student      | E14 E6                 |             |              |

professional or academic representation of psychology; representatives of the student collectives participating in psychology demonstrations; and so on). The interest here lay in complementing the public discourse with the private one, since, given that the latter was more informal, we considered it would help us overcome some of the former's limitations and, for example, provide a better insight into largely subtle inter- or intraprofessional tensions. The interviews were held between October 2008 and June 2014 in the Spanish cities of Madrid, Guadalajara and Barcelona. The average interview time was 75 minutes, with a minimum of 45 minutes and a maximum of 2 hours. They were all recorded and transcribed for analysis.

The selection process of the group of interviewees also attempted to cover the institutional sectors involved in the conflict (professional, academic, and student) as well as the main inter- or intraprofessional or academic distinctions (psychologist, physician–psychiatrist; clinical, educational, and social academic) (Table 2). The interviewees

were primarily selected on the basis of the need to expand or improve discursive understanding (ambiguities, gaps, contradictions, and lack of information) arising from the document analysis. Consequently, the saturation point for the required number of interviews was determined by sufficient discourse redundancy, which enabled us to specify the number of discourses or modes of enunciation in the document analysis. Finally, the interviews also helped to corroborate the collectives' integration in the table of 'modes of enunciation' (professionalizing, cultural, scientific, and political–economic) resulting from the discursive analyses, which we comment on in depth in the following section.

Besides the public documentation and the interviews, the discursive analysis is supported by our previous historical-legal document study of Spanish healthcare regulation (1986–2015) (Rodríguez-López 2015). This research helped us understand the long period of inter- and intraprofessional negotiations and conflicts in the Spanish healthcare sector throughout its regulation

phases. Consequently, the current discourse can be analysed considering material and discursive tensions that not only appeared after the LOPS.

All the material was analysed following the considerations and mechanics of critical discourse analysis (CDA) and on the basis of multiple readings and coding of the material at various points in time. The decision was made not to use qualitative analysis software in the process, as no suitable software was identified for the contextual demands of CDA. CDA differs from other discourse analysis perspectives due to its threefold material insertion: social (institutional), historical (the discourse unfolds over time), and political (link to power conflicts, logics of domination, or ideology). CDA also supports the relational or dialectical nature of discourses (in our case, dialectics between professional discourses) and reclaims the importance of the performativity of language, in other words, the ability language and discourses have to not only reflect reality, but also to construct it (Fairclough 1995; Wodak and Meyer 2003; Leitch and Palmer 2009).

The topics covered in the documents were coded in a first reading to identify purely empirical elements in the texts (Finkel, Parra and Baer 2008). The purpose of the second reading of the documents was to identify comments on the LOPS, a positive or negative evaluation of its content and the criteria used in this evaluation (sector modernization, improvement in interprofessional organization, user protection, exclusion of some professions from recognition, excesses in the regulation of professional practice, and so on). In a third reading of the document content, we identified aspects related to the positive or negative evaluation of psychology as a healthcare profession, as well as the criteria used (adequate or inadequate training processes, evaluation of the scientific or non-scientific nature of the practice, previous credentials, and so on.) Repeated readings determined the main elements enabling us to discern differentiated discourses in the LOPS-psychology discourse as a whole and to determine the collectives within them. After establishing the main discourses, their internal coherence was analysed to find possible distinct discursive modes within them (Fairclough 1995). The internal coherence of each mode was again reviewed until inflections or new modes were ruled out. The final step was coding using conceptual categories taken from the field of study of the sociology of professions. The initial empirical elements of the texts were recoded on the basis of these categories.

Some studies have analysed the legitimization of professional activity based on the rhetorical analysis of the discourses (Siebert, Martin and Simpson 2020). Others have analysed interprofessional conflicts from this same discursive-rhetorical perspective to understand them in a

more dynamic and critical manner rather than as simple institutional specifications (Fincham and Forbes 2019). Fewer studies have conducted research on the organizational or the sociology of professions fields from the CDA perspective (Vaara and Tienari 2002). We have not found any literature that has analysed interprofessional relations from this perspective. Our particular interest in CDA is that this analytical perspective considers the discourse as inescapably associated with specific production contexts and not as isolated objects, and this is the clearest difference compared with traditional linguistic studies (Van Dijk 2000). Therefore, we could view the dynamics of power or the varying relations of domination and resistance pervading the discourses of social conflict as a special consideration of the text-context relationship (Wodak 2003). Consequently, we can complement prior studies containing more in-depth analyses of the Spanish public healthcare sector's transformation and adjustment to new public management, which in this paper form the discursive production context of legitimization (or delegitimization) strategies of the professionalization process (Rodríguez-López 2015).

## FINDINGS

Based on our research, we have identified two clearly differentiated discourses and four modes of enunciation (two modes for each discourse). The specific evaluative stance towards the LOPS allowed us to discern the fundamental division between the materials more clearly, which is why we differentiated, in the first instance, between a discourse opposing the LOPS and another discourse defending it. However, although both defending and opposing the LOPS coincided internally in the fundamental points of enunciation, they nevertheless started from discursive positions that merited a new differentiation. Therefore, in the end, we identified four modes of enunciation, which we have defined as professionalizing (anti-corporate), cultural (corporate), scientific (modernizing), and political-economic. The balances between these modes enabled us to recognize the differing trends and significance of specific formats in the professionalization strategies evident in the conflict. The four modes of enunciation and their main characteristics are presented below. We can also see how the analysed groups are integrated differently in the modes (Table 3).

### Professionalizing discourse (anti-corporate)

The value of the LOPS is defended, insofar as it consolidated the healthcare (and therefore scientific) recognition of the speciality of clinical psychology.

**Table 3.** Modes of enunciation (and associated collectives) in the psychology-LOPS conflict.

| In defence of the LOPS                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | Opposition to the LOPS                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 1) Professionalizing (Anti-Corporate)<br><i>COP Andalucía Occidental [Official College of Psychologists of Western Andalusia]</i><br><i>COP Galicia [Official College of Psychologists of Galicia]</i><br><i>Asociación Española de Psicología Clínica y Psicopatología (AEPCP) [Spanish Association of Clinical Psychology and Psychopathology]</i><br><i>Asociación Nacional de Psicólogos Clínicos y Residentes (ANPIR) [National Association of Clinical and Resident Psychologists]</i><br><i>Mesa Galega de Psicoloxía Clínica (MGPC) [Galician Board of Clinical Psychology]</i>                | 2) Cultural (Corporate)<br><i>Foro de la Psicología de España [Spanish Psychology Forum]</i><br><i>Consejo General de Colegios Oficiales de Psicólogos (CGCOP) [General Council of Official Colleges of Psychologists]</i><br><i>Conferencia de Decanos de Psicología de las Universidades Españolas [Conference of Deans of Psychology of Spanish Universities]</i><br><i>Colectivo de Estudiantes de Psicología (CEP-PIE) [Psychology Students' Collective]</i><br><i>Movimiento contra la LOPS [Movement against the LOPS]</i><br><i>Grupo por la salud y la psicología [Group for Health and Psychology]</i><br><i>Asociación de Psicólogos Afectados/as por el Reconocimiento de la Especialidad en Psicología Clínica [Association of Psychologists Affected by the Recognition of the Speciality in Clinical Psychology]</i><br><i>Federación Española de Asociaciones de Psicoterapeutas (FEAP) [Spanish Federation of Psychotherapists' Associations]</i> |
| 3) Scientific (Modernizing)<br><i>Sociedad Española de Psiquiatría [Spanish Society of Psychiatry]</i><br><i>Consejo General del Colegio de Médicos (CGCM) [General Council of the Medical Association]</i><br><i>Consejo Asesor del Ministerio de Sanidad (CAMS) [Advisory Council of the Ministry of Health]</i><br><i>Comisión Nac. Promotora de la Especialidad de Psic. Clín. (CNEPC) [National Committee for the Promotion of the Speciality of Clinical Psychology]</i><br><i>Comisión Técnica de Formación Especializada (CTFE) [Technical Committee for Specialized Training]<sup>a</sup></i> | 4) Political-economic<br><i>Asociación Española de Neuropsiquiatría (AEN) [Spanish Association of Neuropsychiatry]</i><br><i>[social, educational or critical psychologists]</i>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |

<sup>a</sup>Both the CNEPC and the CTFE are advisory committees of the Ministry of Health.

We think the LOPS is a fantastic Act ... and what it does is lend legitimacy to clinical psychology. In other words, we are professionals, and our role in the system is now defined here. (E6, Professional Psychologist, Chair of Asociación Nacional de Psicólogos Clínicos y Residentes—ANPIR, National Association of Clinical and Resident Psychologists)

This Act also reinforces the rigorous training channels that lead to obtaining the qualification of specialists and to its recognition as health care. All this implies an objection to recognizing psychologists who only hold a bachelor's degree as healthcare professionals, as this could downgrade that recognition and devalue the professional aspect of health psychology practice.

There are legitimate reasons for not recognizing as specialists a number of professionals who do not meet the requirements. In no other profession do rank-and-file graduates hold the same positions as specialists. (Asociación Española de Psicología Clínica

y Psicopatología—AEPCP, Spanish Association of Clinical Psychology and Psychopathology).

The primary issue with the LOPS is the demand to add more places to access PIR training, as the number available is considered insufficient to meet the high demand from graduate psychologists (and considered discriminatory compared to the much more balanced ratios of resident medical interns) as well as, they posit, for the population's actual requirements for psychological care.

As we can see (Table 3), this mode is covered by associations of clinical psychologists or psychologists who are in the process of PIR training and who have already passed the public competitive examinations (known as ANPIR). There are also some regional associations of psychology. In this case, these associations have been critical of the strategies pursued by the official national association, the Consejo General de Colegios Oficiales de Psicólogos (CGCOP, General Council of Official Colleges of Psychologists). The latter, as explained below, is in another mode of enunciation.

This is mainly a discourse defending the channels of rigorous professionalization for healthcare psychology (longer training period with a residency) as provided for by the LOPS. Despite being defended by psychology itself, or at least part of it, the discourse is closer on this point to the discourse of certain medical associations.

This approach shows a marked desire to professionalize psychology, but it also demands a high degree of rigour in its training, often converging with the demands of physicians rather than with those of official psychology. We thus characterize this discourse as professionalizing but anti-corporate, insofar as it opposes the dominant strategy in the discipline, ostensibly advocated to broaden the professional recognition of most of its graduates.

### Cultural discourse (corporate)

This is the dominant discursive approach and strategy in the psychology discipline, or at least the one followed by its main representative bodies: the Spanish Psychology Forum; the CGCOP; the Conferencia de Decanos y Decanas de Psicología de las Universidades Españolas (CDP, Conference of Deans of Psychology of Spanish Universities); and the Colectivo de Estudiantes de Psicología (CEP-PIE, Psychology Students' Collective). In this case, it was argued at the time that obtaining a degree should suffice for the graduate to be recognized as a healthcare professional, as it was for physicians, veterinarians, dentists, and pharmacists.

The psychology degree curriculum adapts to the provisions in article 2.1 of the LOPS. Therefore, amending the Act to incorporate [general] psychology as a regulated healthcare profession is appropriate and justified. (CDP)

To this end, the collectives in this discursive mode advocated for the biopsychosocial aspect of health. In other words, the different forms of psychology (social, educational, etc.) should form part of healthcare training since the latter needs to be approached holistically.

Excluding psychology graduates from healthcare professions stems from a Ministry stance ... that regards healthcare as extremely restrictive and that strays from widely shared views on the importance of social and healthcare issues in developing preventive and treatment policies. (President of CGCOP)

The demands are thus clearly corporate, defending, above all, the space to professionalize psychologists as a whole, without giving preference (at least ostensibly)

to a clinical line associated with a specific part of them. We consider this discourse to be 'cultural' insofar as it reproduces a generalized conception among the population of a substantial (albeit diffuse) confluence between psychology and health (see, for example, the analyses of Buela-Casal et al. 2005).

### Scientific discourse (modernizing)

This approach defends the LOPS as the Act represents considerable progress in the professional regulation of the healthcare sector, in adapting it to the international healthcare situation in the twenty-first century, and in consolidating the scientific aspect of its practice. It mainly includes collectives of physicians and psychiatrists, but also, as was to be expected, groups or bodies from the health authority or its advisors (Consejo Asesor del Ministerio de Sanidad—CAMS, Advisory Council of the Ministry of Health; Comisión Nacional Promotora de la Especialidad de Psicología Clínica—CNEPC, Spanish National Committee for the Promotion of the Speciality of Clinical Psychology); and Comisión Técnica de Formación Especializada—CTFE, Technical Committee for Specialized Training) (see Table 3). This discourse thus forms the orthodox and legitimizing perspective of the very bodies of political government implementing the new law.

The healthcare system needed to be modernized to arrive at a more open model, not only for the profession but for professional activity in the healthcare system, and to bring it into line with what I believe is a successful model: the Anglo-American model. (E1, Professional Physician, member of the drafting team for the draft bill of the LOPS)

Also noteworthy in this mode is the position of the CNEPC. This Committee, comprising members of the main clinical psychology associations, differs as a whole from the latter's discourse (mode) by defending the new Act using an 'institutional' discourse of the sector rather than 'internal' approaches to psychology (necessary professionalization, rigorous training, etc.), thus stressing the need for scientific consolidation of the sector's practices and the response to the various needs to modernize it. This also involves not considering psychology (at degree level) as a healthcare profession.

There is a clear position, shared by all healthcare administrations, of not considering psychology as a qualified, regulated profession, as they deem that clinical subjects are not given enough significance in the psychology syllabus. (CNEPC)

### Political–economic discourse

The last mode represents opposition to the LOPS. Although the stance generally defends health psychology, rather than being recognized in this debate, it is viewed as referring the conflict to a broader, political–economic problem lying at the root of this Act. Above all, we are referring to the criticism levelled at introducing new public management into the healthcare framework, especially through the precarization of the professional work that may result from this regulation.

There are no psychological collectives (as such) in this space. We have not found any pockets of critical positions in them, highlighting the political–economic issues underpinning the new legal–healthcare aspects analysed—at least never beyond specific personal views and voices that find it hard to participate in the prominent spaces of public debate. In this case, they have mainly been individuals (hence their insertion in brackets in Table 3) who are generally linked to social, educational, or critical psychology. Neither are there structuring discursive perspectives in this regard in the student movements opposing the Act (CEP-PIE and the Anti-LOPS Movement). Instead, they have unified their collective voice with the main professional (CGCOP) and academic (CDP) bodies, generally concerned about opportunities in their professional future.

The CGCOP's stance is the same as ours because we share the same interests. (CEP-PIE)

Also of interest is the fact that statements made by groups in the physician–psychiatrist field fall into this mode of enunciation. These groups include the Asociación Española de Neuropsiquiatría (AEN, Spanish Association of Neuropsychiatry) and even the Consejo General del Colegio de Médicos (CGCM, General Council of the College of Physicians). However, the latter has not been included in this mode of enunciation, since few statements fall into it, and they do not saturate a structured, differentiated and productive mode of discourse. Instead, they form a small set of critical reflections merely accompanying or clarifying generic acceptance of the LOPS.

Alongside the introduction of new employment and hiring guidelines... exercising professional ideology has suffered enormously, as it has to be exercised in temporary, part-time and poorly remunerated work contexts. (CGCM)

### DISCUSSION AND CONCLUSIONS

Below, we interpret our results and revisit the three levels underpinning our analysis. Firstly, we outline the

inter- and intraprofessional tensions evident after the healthcare sector regulations analysed above and psychology's position as a result of them. Secondly, we analyse the discursive professionalization strategies used by Spanish psychology to defend its professional importance in the sector. This also provides us with insights into the specific professionalization moment or phase of Spanish health psychology and helps us compare it with studies on other healthcare professions. Lastly, we review the extent to which the political–economic context impacted the psychology discourse. We also consider these three levels of analysis within the international context of healthcare regulations and of the conflicts these regulations have caused with psychology in other countries.

In light of our results, we must rule out a simple reading of the LOPS as an interprofessional conflict between psychologists and physician–psychiatrists, which some of those involved in the conflict seemed to suggest. That is why we defined one of the discursive modes in our results as 'anti-corporate', since it includes numerous psychology collectives that did not follow the discursive line of the main professional and academic representative bodies. This discursive mode is saturated by clinical psychologists. Some clinical psychology associations have argued, generically, that not all of psychology as such was health care. We therefore contend that the most outstanding professional achievement and regulation in the recent history of psychology in the clinical field—inclusion in 1998 in the Spanish Health System through the specialist route of the 'Official Qualification of Psychologist Specializing in Clinical Psychology'—has become, over time, a paradoxical professionalizing success because it has led to a split, or at least to central internal conflicts, within the discipline. It is highly significant that the discourse content of these clinical (psychology) collectives is now similar to the discourses psychiatrists used to oppose (clinical) psychology entering the public healthcare system in 1998 (Rodríguez-López 2015). Nevertheless, also relevant in this case is the fact that the main bodies representing psychology in Spain favour an interpretation of 'healthcare' within this conflict that is at odds with the European context (EuroPsy Certificate) because the EuroPsy requires a longer and more specialized training.

Our results are consistent with those of other studies on professionalization processes in the healthcare sphere that have evidenced internal divides in professions during these processes. These divides have led to both interprofessional and intraprofessional conflicts as differing professionalization strategies have been followed, as evidenced among paramedics (Brydges 2022). Furthermore, our results are consistent with the dual closure put forward by authors such as Witz (1992) and

King et al. (2015) in their studies on boundary conflicts in healthcare contexts. Just like nursing staff, psychologists attempt to expand their legitimate sphere of practice into the field traditionally reserved for physicians; however, at the same time, they try to protect their own boundaries from being usurped by other less recognized disciplines. As in the documented case of chiropractors (Brosnan 2017), these internal divides in the profession could lead the clinical part of psychology to align more with the physicians' discourse, for example, in demanding longer training processes. However, part of psychology has resisted joining the traditional medical model. This is more evident in psychology's resistance in the UK and France. This part of psychology (which could be represented by collectives such as Federación Española de Asociaciones de Psicoterapeutas—FEAP—Spanish Federation of Psychotherapists' Associations—in Spain) has barely managed to achieve a presence of any relevance in the public debate here with the LOPS. This type of intraprofessional differentiations could perhaps be seen as a struggle to evaluate various forms of capital (Bourdieu 2009) within the field of psychology (for example, 'scientific-technical' compared with 'social-health' or 'patient-centred'), in a way that is similar to the findings of Brosnan (2017) and Schinkel and Noordegraaf (2011). In fact, Bourdieu's perspective, conceptualizing professions as fields, could prove relevant for future studies of this type of conflict and negotiation in the sector.

Concerning the comparison of psy resistance discourses in the UK and France with those analysed for Spain, we cannot contrast them directly. This is because we have shown an internal split in psychology, resulting in opposing discourses within the same profession. In previous studies (Rodríguez-López 2012, 2015), we have seen that the objectives of healthcare regulations that have changed the professional structure in the public sector in Spain are very similar to the ones outlined in this article for the UK and French cases. However, the resistance processes and critical discourse of psychology differ. As we have seen, the central criticism of psychology in the UK and France to implicit medicalization in healthcare regulations does not appear directly here. However, there is one important point in common: the one argued by the official representatives of Spanish professional and academic psychology (CGCOP, CDP, CEP-PIE), and the criticism, especially in the UK, that the standards provided by the new regulations prefer a specific type of therapeutical perspective. This would involve a restricted concept of 'healthcare' and, therefore, it would negatively impact the part of psychology whose working perspectives are not directly aligned with medical practice (in Spain, non-clinical psychology). However, our research

has some limitations in this regard, as the depth of analysis of the UK and French cases is not as great as in the analyses for the Spanish case. A deeper discourse analysis of UK and French psychology struggles could perhaps find fragmentation or dual closure processes similar to those found for Spanish psychology. More case and comparative studies are necessary to explore our results further.

Furthermore, we also want to highlight the methodological interest in analysing the discourse on the conflict using the actual content of the discourse and not from an a priori differentiation of the professions or their collectives (Bucher et al. 2016), since the former allows us to more clearly identify possible intraprofessional distinctions or even, as in our case, discourses or discursive modes that include a variety of professional associations. Similarly, the discursive analysis complementing the public documentation of professional collectives with the private discourses of some of their representatives makes it possible to overcome the limitations of only analysing public documents (Bucher et al. 2016), since their formality can occasionally render it more complicated to find subtle inter- or intraprofessional tensions, which became far clearer in the personal interviews in our case.

From a historical perspective, the professional integration of psychology in the public health sector in Spain, by creating the 'Psychologist Specializing in Clinical Psychology' qualification in 1998, is relatively recent. In terms of professionalization strategies and possibilities, the position of (clinical) psychology in the sector is mostly similar to nursing (due to its conflict with mainstream medicine, demands for its value for the sector to be recognized, cultural support from users, defence of its university education, and its scientific quality, etc.). However, sometimes psychology's strategies are also unexpectedly similar to 'paramedical' or alternative medicine professions (Saks 1992; Brosnan 2017), as some debates focus majorly on epistemological aspects and the varying degree of the discipline's scientific validity as well as the possible extent of its healthcare aspects. The latter reminds us of an issue that used to occur in previous professionalization phases. This is striking in a profession that has been recognized in the sector for at least a couple of decades.

Finally, we refer to the more general level of consequences of the healthcare regulation concerning its possible political-economic effects.

First, the processes, discourses and legal amendments to the analysed professional healthcare regulation in Spain converge in significant ways with those observed in other countries in the Western world, such as the UK, USA, France, Canada, and Australia (Champion 2006; Rodríguez-López 2015; Leslie et al. 2021; Rowland,

Albert and Kitto 2021; Adams 2022). This shows the global reach of the transformations underway. However, we realize the need to broaden cross-national studies, as stated in other papers (Leslie et al. 2021; Adams 2022). We would further add that more non-Anglosphere evidence is required, since Anglo-American countries have the highest volume of studies in this regard.

There are no references to hybridization processes in the discourse of Spanish psychology concerning these new regulations. This prevents us from drawing conclusions from our results on how these hybridization processes affect Spanish health psychology. Could this mean that psychology is not affected by new public management? That is highly unlikely. In fact, it is impossible for psychology not to be affected by new public management, since this affects all healthcare professions in the public sector. In our view, the structural position of the profession in the healthcare context (peripheral and low-status) leads to the advantages of its professionalization process discursively outweighing the possible loss of previously established professional privileges in the sector or of the effects of a possible hybridization of its professional role. Consequently, physician-psychiatrists (centrality and high status) more often mention the consequences of these organizational transformations. Nevertheless, it is clear that many questions require either more detailed linguistic analysis at the 'textual' level (perhaps with the detail afforded by some qualitative analysis software) or broader societal analysis at the 'social-cultural' level in order to integrate these two CDA dimensions in more detail. In future research, it would be interesting, therefore, to explore whether psychology is actually pursuing hybrid positions in its professional healthcare work. If this is the case, the analysis should focus on whether the discursive absence of the issue evidenced herein has resulted from structural invisibility or whether it is a deliberate strategy.

Furthermore, in clear contrast with the context of resistance and confrontation of psychology with the new regulations in other European countries, such as the UK and France, a political-critical reading of the new regulations is almost absent in Spain. The position of associations, such as the AEN, is noteworthy in this regard. As a result of a cross-discipline discourse and broad sensitivity to the macro-sociological determinants of health, they can promote interdisciplinary workspaces without losing sight of the political-economic consideration of transformations affecting healthcare. This provides us with some insights into the value of associations that are interprofessional rather than purely corporate and that can more easily perceive transformations having a similar impact on healthcare professions.

Finally, the importance of university students—future psychology professionals—should not be omitted. In our research, we have observed how they align with the main representatives of professional and academic psychology. However, it is highly plausible that significant legal transformations in the health sector, coupled with its inter- and intraprofessional conflicts, are shaping the professional expectations, careers and identities of new generations. On the basis of our results, an interesting future line of research would involve analysing whether the tensions evidenced for clinical, cultural and political-economic discourses are reproduced or overcome in the new cohorts.

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